



SEDATIVE PHYSIO

(Registered under Govt. of India)

Subject – PT Neuro

Topic – Stroke PT Management

Goals of PT Management

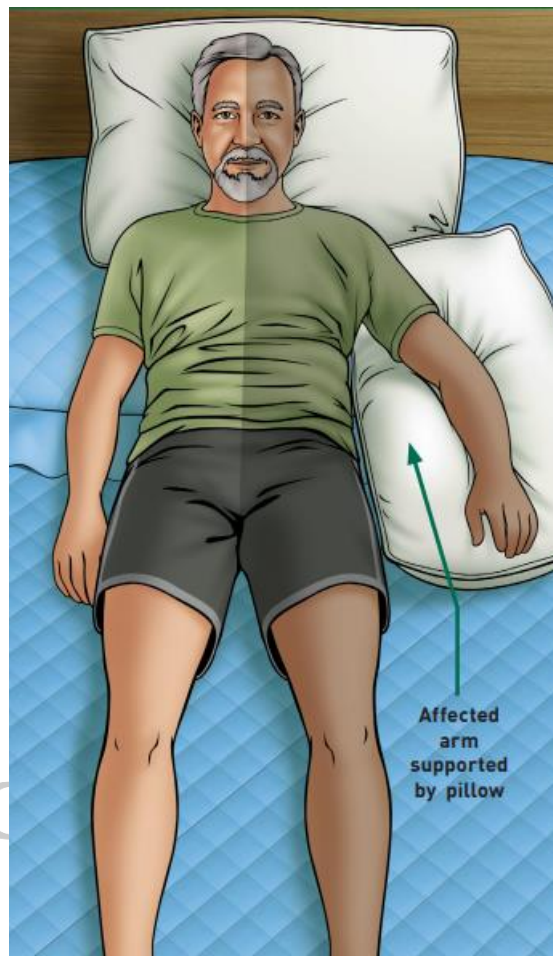
- Regain motor function
- Regain sensory function
- Improve muscle strength & performance
- Improve gait
- Improve balance & coordination
- Improve joint flexibility & mobility
- Improve ADL
- Prevent further complication

Management

Positioning

- **Positioning is used to:**
 - Provide support
 - Prevent abnormal muscle tone
 - Improve alignment & weight bearing
 - Provide comfort
 - Prevent pressure sores
 - To increase awareness of the affected side.
- **Supine**

- Should be used with prevention
- High risk of pressure sores
- Head should be in midline on pillow
- Place towel under affected scapula (shoulder jt.)
- Affected elbow should be extended
- Place pillow below affected forearm
- Place rolled pillow affected pelvis



- **Side Lying (Unaffected Side)**

- Head supported on pillow
- Bring affected shoulder forward
- Place pillow below the affected arm
- Affected hip and knee flex
- Place pillow below the affected knee joint

- Keep the spine of the patients straight



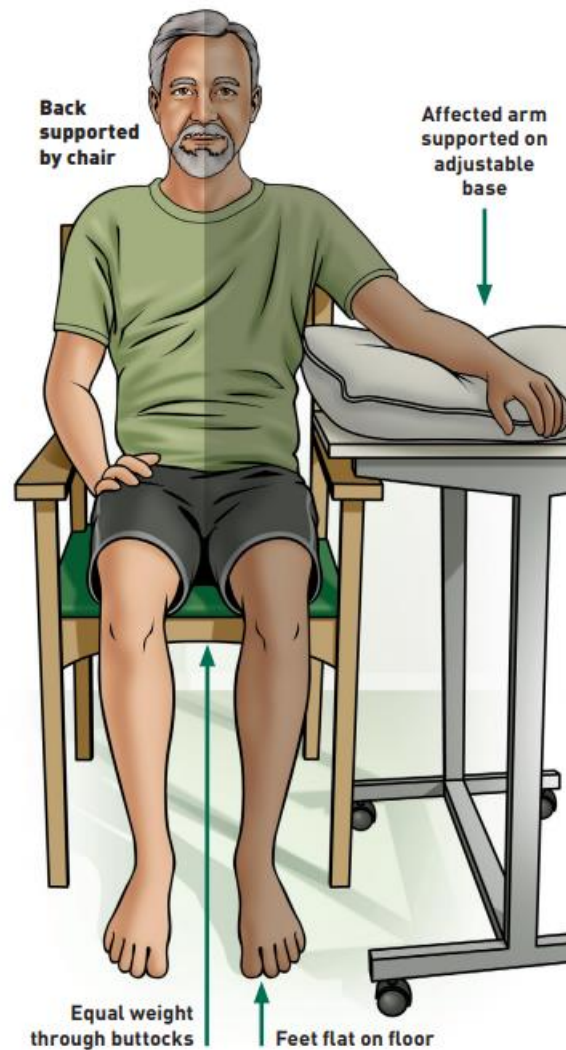
- **Side Lying (Affected Side)**

- Head supported on pillow
- Affected shoulder placed on pillow
- Unaffected leg forward with hip and knee flexed
- Affected leg straight
- Place one pillow in front and one in back of the patient



- **Sitting**

- Patient should sit upright
- Head should be in midline
- Prefer chair with handrest
- Symmetrical weight bearing on both buttocks
- Hip and Knee flexed to 90 degrees
- Ankle neutral



Motor Learning Strategies:

- Scapular mobilization
- Touching opposite shoulder (Initially passively. Go towards active movt. as per the recovery)
- Touching the head (Initially passively. Go towards active movt. as per the recovery)
- Bridging Activities
 - Full Bridging
 - Half Bridging
 - With Affected leg
 - With Unaffected leg

- Passive movement of all joints
- PNF Patterns
- Trunk Rotation
- **Activities in sitting**
 - Weight bearing on affected limb
 - Therapist will stand behind patient
 - Put the patients arm in slight sha bd and elbow extension
 - Ask the patient to bear weight on the arm
 - Shoulder shrugs
 - Touching opposite shoulder
 - Touching sacrum
 - Forearm movement (Passive/Active)
 - Knee Joint movement (Passive/Active)
 - Sit to Stand transfers
 - Therapist will stand in front of patient
 - Therapist will stabilize both pelvis
 - Patient's both arms should be on patient's shoulder
 - Help the patient to stand
 - Physio ball exercises
- **Activities in standing**
 - Putting the leg forward
 - Therapist will stabilize the patient's pelvis
 - Therapist will then ask the patient to place his affected leg forward and take back.
 - If patient can't do this actively, then passively this should be done.
 - Putting leg laterally (Abduction & Adduction)

- Therapist will stabilize the patient's pelvis
 - Therapist will then ask the patient to place his affected leg laterally (sideways) and take back.
 - If patient can't do this actively, then passively this should be done.
- Unilateral stance on affected leg
 - Pivot turning

Gait Training:

- When the patient becomes able to do the transfers actively specially in the standing position, then we can proceed for gait training.
- Parallel bars
- Progress by removing the bars
- Side stepping
- Backward stepping
- Pivot turning
- Treadmill walking

Balance:

- Quadruped position
- Superman position
- Kneeling
- Knee standing
- Kneeling to knee standing
- Reaching activities in knee standing
- Reaching activities in kneeling
- Balancing board exercises

- Single limb standing
- Toe standing
- Physio/Swiss ball exercises

Joint flexibility, Mobility & Strength

- Soft tissue manipulation
- Peripheral joint mobilization
- Passive Movement
- Stretching
- Gradually move towards active movement
- Then move towards the active resisted movements
- Positioning
- Splinting
- Slings
- Weight bearing exercises
- PRE
- Wall squats
- PNF patterns with resistance

Some other approaches/techniques

- Neurodevelopment Technique (NDT)
- Rood's approach
- PNF Technique
- CIMT